

1. Module 2: Part C and Other Medicare Health Plans

2. Navigation Instructions

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4. Learning Objectives

- After reviewing “Part 2: Medicare Health Plans” you will be able to:
 - Explain what types of Medicare health plans are available
 - Explain who is eligible for the different types of plans
 - Understand the role of Special Needs Plans (SNPs) in delivering care
 - Describe features of different Medicare health plan types
 - Describe key issues for beneficiaries eligible for both Medicare and Medicaid
 - Explain how Medicare health plans work with prescription drug plans

5. Training Roadmap: Module 2

- Medicare Advantage Plans
- MA Plan Types: Coordinated Care Plans
- Special Needs Plans
- MA Plan Types: Private Fee-for-Service (PFFS) Plans
- MA Plan Types: Medical Savings Account (MSA) Plans
- Medicare Advantage Employer/Union Plans
- Medicare Advantage Plans and Dual Eligible Beneficiaries
- Medicare Advantage and Prescription Drugs
- Other Type of Medicare Health Plans
- Enrollee Protections: Appeals and Grievances

6. Title Page: Medicare Advantage Plans

7. Part C: Medicare Advantage Plans: Overview

Under the Medicare Advantage (MA) program, known as Medicare Part C, private companies offer health plans that cover all Medicare Part A and Part B benefits.

- Many also cover Part D prescription drug benefits (MA-PD plans)
- All MA plans have a maximum out-of-pocket limit (MOOP) for basic benefits
- Many MA plans also offer additional benefits that Medicare does not cover
- The types of Medicare Advantage (MA) plans are:
 - Coordinated Care Plans. These plans have a network of preferred providers and include:
 - Health Maintenance Organizations (HMOs), some have a point-of-service (POS) benefit that allows beneficiaries to go out-of-network subject to limitations
 - Preferred Provider Organizations (PPOs), which may be local or regional
 - Private Fee-for-Service (PFFS) Plans
 - Medical Savings Account (MSA) Plans

8. Medicare Advantage Eligibility

To be eligible to enroll in a Medicare Advantage plan:

- A beneficiary must be entitled to Part A ***and*** enrolled in Part B.
 - The beneficiary must permanently live in the MA plan's service area. (If a beneficiary spends six months or more outside of the plan's service area, he or she should only enroll in MA-PD plans with a visitor/traveler benefit.)
 - Be a U.S. citizen or lawfully present in the United States on or before the enrollment effective date. (CMS makes this determination)
- Certain MA plans including MA MSA plans, Special Needs Plans, and Employer Group Waiver Plans have additional eligibility requirements.

9. Medicare Advantage Eligibility, continued

MA plans must enroll any eligible beneficiary who applies regardless of health status.

- Beginning in 2021, beneficiaries with ESRD became eligible to enroll in Medicare Advantage plans as long as they met other eligibility requirements.
- Certain special needs plans (SNPs) known as chronic condition SNPs or C-SNPs can limit enrollment to beneficiaries with certain chronic conditions.

10. Medicare Advantage Plans: Premiums

Medicare Advantage Plans may charge a premium. If the plan charges a premium, beneficiaries must generally continue paying their Part B premium in addition to paying the monthly plan premium to remain enrolled.

11. Medicare Advantage Plans: Cost-Sharing and Maximum Out-of-Pocket Limits

- Medicare Advantage plans may also require their members to pay for a portion of the covered services they receive. This is known as member cost-sharing. There are several potential types of cost-sharing:
 - Deductible: A set amount the member must pay for covered services before the health plan begins paying for those services.
 - Copayment: A fixed dollar amount per service the member must pay. For example, \$10 for each primary care visit.
 - Coinsurance: A percentage of the cost of the service the member must pay.
- All Medicare Advantage plans must have a “maximum out-of-pocket” limit (known as the “MOOP”) for Part A and Part B benefits. That is, once the member pays a specified amount of cost-sharing, the health plan covers 100% of covered medical services. Each year CMS specifies a mandatory MOOP, which health plans cannot exceed, although they may have a lower MOOP.
 - Each plan’s MOOP will be specified in its summary of benefits and its evidence of coverage.

12. Part C: Medicare Advantage Plan Benefits

- All Medicare Advantage (MA) plans must cover all Part A and Part B benefits.
- Most Medicare Advantage plans also cover all or a part of the Original Medicare cost-sharing for Part A and Part B benefits.
- Medicare Advantage plans may also cover extra benefits not covered by Original Medicare, such as:
 - Additional days of hospitalization
 - Skilled nursing and rehabilitative services without a prior 3-day inpatient hospital stay
 - Vision Services, including glasses
 - Hearing Aids
 - Routine Dental Services and/or Dentures
 - Fitness (such as gym membership or Silver Sneakers)
 - Meals related to a medical condition or after a hospitalization
 - Worldwide Urgently Needed and Emergency Services
 - Over the Counter Drugs

* An annual physical is different from the annual wellness visit covered under Medicare, which does not include a physical exam.

13. Special Benefits Depending on Chronic Health Condition

Medicare Advantage plans may offer special benefits for individuals with certain chronic health conditions, such as diabetes, heart failure, COPD, or other conditions, that are not available to members of the same plan without the specified condition. There are two categories of such benefits: (1) those that are primarily health related and (2) those that are not (the latter generally address social determinants of health and are known as Special Supplemental Benefits for the Chronically Ill, or SSBCI).

- Primarily health related benefits for chronically ill enrollees may include items such as: decreased cost sharing for certain services, or supplemental benefits such as at-home palliative care or transportation to medical appointments.
- SSBCI may include items such as: food and/or produce, meals beyond a limited basis, pest control, and non-medical transportation.

Consequently, it is useful for agents to know if their clients have conditions that may qualify them for these types of benefits if they are available in the clients' area.

14. Medicare Advantage Plan – Utilization Management

Medicare Advantage plans may implement mechanisms to manage the utilization of covered services that do not apply under Original Medicare.

Such mechanisms include requiring a referral or prior authorization to obtain a service.

Plans may also implement step therapy requirements for Part B or Part D drugs. Step therapy is when a plan requires a beneficiary to try less expensive options before "stepping up" to drugs that cost more.

15. Title Page: MA Plan Types Coordinated Care Plans

16. MA Plan Types Coordinated Care Plans – HMOs

- HMO enrollees must generally only use doctors and hospitals within the plan's network (known as participating providers) for services to be covered. However, there are certain exceptions:
 - Emergency services received outside of the plan network are covered.
 - When the enrollee is temporarily absent from the plan's service area, dialysis services are covered outside of the network.
 - Urgently needed services received outside of the plan network are covered when the enrollee is temporarily outside of the service area or in rare circumstances when the network is not available.
 - If a needed specialist or a covered procedure is not available through the network, the plan will authorize out-of-network services.
- The mandatory MOOP for HMOs in 2023 is \$8,950, although it is likely that many will have lower limits.
- HMO enrollees may need to select a primary care doctor and may need a referral for specialty care.

- If an HMO enrollee needs a type of specialist who is not available in the plan's network, the plan will arrange for care outside of the network.
- Some HMOs offer a Point of Service (POS) option that allows enrollees to go to non-plan doctors and hospitals without receiving prior approval for certain services.
 - Unlike a PPO, an HMO-POS plan may limit the services available out of network or may put a dollar cap on the amount of out-of-network coverage.
 - Cost-sharing is generally higher than for services obtained from network providers.

17. MA Plan Types Coordinated Care Plans – PPOs

Under a PPO, enrollees:

- may get care from any provider in the U.S. who accepts Medicare; they are not limited to network providers.
 - do not need a referral to see an out-of-network provider but are encouraged to contact the plan to be sure the service they wish to obtain out-of-network is medically necessary and will be covered.
 - usually pay higher cost-sharing amounts than they would pay in-network if they see an out-of-network provider.
- PPOs must have a maximum limit on member out-of-pocket costs for network providers of not greater than \$8,950 in 2023 although it is likely that many will have lower limits. They must also have an aggregate MOOP for network and non-network providers of \$12,450 in 2023. Again, it is likely that many will have lower limits.
 - Regional PPOs are PPOs that are offered throughout an entire region, made up of one or more states.

18. MA Plan Types Coordinated Care Plans – Special Needs Plans

- Special Needs Plans (SNPs) are a type of Medicare Advantage coordinated care plan (HMOs or PPOs) that are specially designed to serve a subset of Medicare beneficiaries.
- In addition to meeting all other MA eligibility criteria, beneficiaries must also meet criteria specific to the type of SNP in which they wish to enroll.
- All SNP plans include prescription drug coverage.

19. Title Page: Special Needs Plans

20. Special Needs Plans – Eligibility (1 of 2)

There are several types of Special Needs Plans

- Chronic condition SNPs (C-SNPs) are SNPs that restrict enrollment to individuals with certain chronic or disabling chronic conditions, such as diabetes, heart failure, cancer, lung conditions, HIV, or certain other conditions.
 - Each C-SNP will specify the condition or conditions necessary to be eligible to enroll.
- Dual eligible SNPs (D-SNPs) enroll beneficiaries who are entitled to both Medicare and medical assistance from a State plan under Medicaid.
 - A subset of D-SNPs, fully Integrated Dual Eligible (FIDE) SNPs, provide dual-eligible enrollees access to Medicare and Medicaid benefits under a single managed care organization.

21. Special Needs Plans – Eligibility (2 of 2)

- Institutional SNPs (I-SNPs) are SNPs that restrict enrollment to MA eligible individuals who, for 90 days or longer, have had or are expected to need the level of services provided in a long-term care (LTC) skilled nursing facility, a LTC nursing facility (NF), an intermediate care facility for the individuals with intellectual and developmental disabilities, a long-term care hospital, an inpatient psychiatric facility, or certain other facilities specified by CMS.
- Institutional Equivalent (IE) SNPs enroll MA eligible individuals who live in the community but require an institutional level of care. Eligibility for an IE-SNP may be limited to certain Assisted Living Facilities.

22. Special Needs Plans – Model of Care

Each SNP is required to develop and implement a “model of care” to address the health needs of their target population. The Model of Care is the SNP’s CMS-approved plan for coordinating care and providing services to its enrollees.

As part of their Model of Care, SNPs must:

- Conduct a comprehensive initial health risk assessment of each enrollee’s physical, psychosocial, and functional needs as well as annual health risk reassessment;
- Manage the care of enrollees using an interdisciplinary team (IDT) that includes providers with applicable expertise and training in treating individuals similar to the targeted population of the plan; and
- Develop and implement a comprehensive individualized plan of care through the IDT, identifying goals and objectives for each enrollee, including measurable outcomes and specific services and benefits to be provided to the enrollee.

23. Title Page: MA Plan Types: Private Fee-for-Service (PFFS) Plans

24. MA Plan Types: Private Fee-for-Service (PFFS) Plans (1 of 3)

- Individuals enrolled in PFFS plans may receive covered services from any provider in the U.S. who is eligible to provide Medicare services and agrees to accept the plan's terms and conditions of payment. They are not limited to a network of plan providers.
- Some PFFS plans contract with network providers. If the PFFS plan has a network, enrollees may pay more if they see out-of-network providers.
- Except for emergencies, enrollees must inform providers before receiving services that they are a PFFS plan member, so the non-network providers can decide whether to accept the plan's terms and conditions.
- Non-network providers that accept Original Medicare may choose not to accept PFFS plan enrollees. Therefore, an enrollee needs to confirm that his or her provider of choice will accept a PFFS plan before enrolling in one.

25. MA Plan Types: Private Fee-for-Service Plans (2 of 3)

- Providers are prohibited from charging a PFFS enrollee more than the cost-sharing specified in the PFFS plan's terms and conditions of payment.
 - Cost-sharing may include balance billing up to 15% of the Medicare rate only if allowed in the plan's terms and conditions of payment.
 - Balance billing happens when a doctor is eligible to accept Medicare but is not a Medicare "participating" provider under Original Medicare. Under Original Medicare, these non-participating providers are allowed to balance bill beneficiaries up to 15% over the Medicare payment amount.
 - PFFS plans may choose whether or not to allow non-participating providers to balance bill their members.
 - PFFS plans must have a maximum limit on member out-of-pocket costs (MOOP) for network and non-network providers of not greater than \$8,950 per year in 2023.
- PFFS plans may choose to offer Part D benefits but are not required to do so.

26. MA Plan Types: Private Fee-for-Service Plans (3 of 3)

Mr. Young wished to enroll in a PFFS plan. Before enrolling, he called to confirm that his doctor would accept the plan. His doctor's office advised him that they would accept the plan, but while his doctor was eligible to participate in Medicare, she was not a Medicare participating provider. Mr. Young checked to see whether his PFFS plan allowed balanced billing before deciding to enroll.

27. Title Page – MA Plan Types: Medicare Savings Account Plans

28. MA Plan Types: Medical Savings Account (MSA) Plans

- A Medicare MSA is a high deductible health plan which is combined with a savings account used to pay for health care expenses not covered under the health plan
 - Medicare contributes to the beneficiary's savings account
- MSA enrollees pay for health care expenses from the savings account and then out-of-pocket until the annual deductible is met, after which the plan pays 100% for covered services
 - The maximum deductible for MSA plans in 2023 is \$15,750, although many MSAs will have a lower deductible.
- MSAs cover Part A and Part B benefits after the deductible
- MSAs do NOT cover, Part D Medicare prescription drug benefits
 - MSA enrollees must enroll in a stand-alone PDP if they want prescription drug benefits
- Enrollees pay their Part B premium and any premium for supplemental benefits

29. MA Plan Types: MSA Plans, continued

- Enrollees may receive covered services from any Medicare approved provider in the U.S.
- MSAs may not have a network or may have a full or partial network of providers.
- All non-network providers must accept the same amount that Original Medicare would pay them as payment in full. This is the amount the enrollee will pay the provider before the deductible is met.
- In 2022, MA MSA plans were only available in 37 states and the District of Columbia.

30. MSA Example

Mr. Jimenez recently enrolled in an MSA from Original Medicare. He was surprised to receive a statement from the plan indicating he had financial responsibility for the screening colonoscopy services he received two weeks prior. He called the plan and said he was confused because most preventive benefits have zero cost sharing under Original Medicare. The plan explained that under an MSA, beneficiaries must pay out-of-pocket for even preventive care services if they have not yet reached the annual deductible.

31. MSAs: Eligibility

The following individuals are **not** eligible to enroll in an MSA:

- An individual who receives health benefits that cover all or part of the annual deductible under the MA MSA plan. Examples include, but are not limited to, primary health care coverage other than Medicare, Medicare hospice, certain supplemental insurance policies, and retirement health benefits.
- An individual who is enrolled in a Federal Employee Health Benefits plan or is eligible for health care benefits through the Veteran's Administration.
- Dual eligibles entitled to coverage of Medicare cost-sharing under Medicaid.

- An individual who cannot provide assurances that he or she will reside in the United States for at least 183 days during the year for which the election is effective.
- An individual who has already elected hospice.

32. Title Page: Medicare Advantage Employer/Union Plans

33. Employer/Union Plans

- Employers and unions may offer their retirees and their dependents:
 - Medicare Advantage individual plans (plans open to the public).
 - Medicare Advantage plans are only available to individuals based on their employer, known as Employer Group Waiver Plans or EGWPs.
 - A Medicare Advantage plan through a direct contract between the employer or union and CMS, known as a direct contract plan.
- Employers with less than 20 employees may be able to offer Medicare Advantage plans to their active employees and their dependents.
- Active employees of employers with at least 20 employees must retain their non-Medicare group health plan because Medicare is secondary for them.

34. Medicare Coordination with Employer Group Health Plans

The rules for what plans employers may offer to their Medicare-eligible employees and whether Medicare or the group health plan pays primary depend on several factors, including:

- whether the coverage is offered by a large group health plan.
 - A large group health plan (GHP) has 20 or more employees for each working day in each of 20 or more calendar weeks in the current calendar year or the preceding calendar year.
- whether the individual has coverage under the individual's or the individual's spouse's current employment.
 - Medicare is primary for *retirees* covered under GHPs.

35. Medicare for Individuals Who are Still Working – Large Group Health Plans

Medicare law prohibits large GHPs from considering that an individual (or the individual's spouse) covered under the GHP by current employment status is entitled to Medicare benefits.

It also requires large GHPs to provide any individual age 65 or older (and the spouse age 65 or older) who has current employment status the same benefits under the plan under the same conditions as any such individual (or spouse) under age 65.

It is illegal for a large GHP to encourage a Medicare-eligible employee to decline the employer GHP and obtain Medicare instead or offer them a different coverage than offered to individuals who are not Medicare eligible. This would include offering its employees and their spouses a Medicare Advantage Plan or a Medicare Supplemental Plan.

36. Medicare for Individuals Who are Still Working – Small GHPs

Medicare is the primary payor for individuals who have group health coverage as a result of their or their spouse's current employment with a company that is not a large GHP. That is, Medicare will pay before the group health plan and the employer does not have to offer an individual who is Medicare eligible coverage under the group health plan.

In addition, small GHPs are not subject to the requirement to offer their age 65 and over employees and their spouses the same benefits under the plan under the same conditions as any such individual (or spouse) under age 65.

Such employers can purchase a Medicare Advantage plan for their employees and their spouses or offer them Medigap coverage.

37. Medicare Advantage Eligibility: EGWPs

- Employer group waiver plans (EGWPs) or direct contract plans may only enroll Medicare beneficiaries who are active employees or retirees of the employer or union offering the plan.
 - A beneficiary's enrollment in an EGWP must be based on receiving "employment-based" health coverage from an employer/union group health plan sponsor.
 - Coverage obtained through a professional or another type of group association would not make a beneficiary eligible for an EGWP, except to the extent that the coverage obtained through the association can properly be characterized as "employment-based" group health plan coverage.

38. Medicare Advantage Eligibility: EGWPs Examples

An association of employers, such as school boards, may offer their former employees an EGWP plan. Employers may form associations for purposes of offering their retirees health coverage.

A professional association, such as a lawyer's association, may not offer an EGWP because membership in the association is based on profession, not the individuals' employer or former employer.

39. Title Page-Medicare Advantage Plans and Dual Eligible Beneficiaries

40. MA Plans and Dual Eligible Beneficiaries

Beneficiaries who qualify for both Medicare and Medicaid are considered "dual eligible" individuals. Dual eligible beneficiaries include beneficiaries enrolled in Medicare Part A and/or Part B and receiving full Medicaid benefits and/or assistance with Medicare premiums or cost-sharing. The Medicaid programs that help beneficiaries pay for premiums or cost-sharing are also known as "Medicare Savings Programs."

These programs generally fall under 4 categories:

- The Qualified Medicare Beneficiary (QMB) program:
 - helps pay premiums, deductibles, coinsurance, and copayments for Part A, Part B, or both programs.
- The Specified Low-Income Medicare Beneficiary (SLMB) program:
 - helps pay Part B premiums.
- The Qualifying Individual (QI) program:
 - helps pay Part B premiums.
- The Qualified Disabled Working Individual (QDWI) program:
 - pays the Part A premium for certain disabled and working beneficiaries.

41. MA Plans and Dual Eligible Beneficiaries (2 of 3)

Qualified Medicare Beneficiaries (QMBs) fall into two categories:

- QMB only – those beneficiaries who just receive assistance paying for their Medicare premiums and cost-sharing.
- QMB plus – those beneficiaries who are also eligible for full Medicaid benefits.

Special rules apply to QMBs:

- When a QMB enrolls in an MA plan, the beneficiary does not have to pay more cost-sharing than any minimal copayment that would apply under Medicaid.
- All providers (whether or not they are Medicaid participating, or in-network) are prohibited by law from balance billing QMBs for any Medicare cost-sharing amounts. Providers who balance bill are subject to sanctions.

Individuals that fall into other categories of Medicare Saving Programs may also be eligible for full Medicaid benefits. Such individuals, along with QMB plus individuals, are known as full-benefit dual eligible (FBDEs).

42. MA Plans and Dual Eligible Beneficiaries, (3 of 3)

- Dual eligible beneficiaries may enroll in any type of MA plan except an MA MSA.
- Some MA plans, known as **dual eligible Special Needs Plans (D-SNPs)**, are tailored to dual eligible individuals, depending on the category to which they belong.
- Issues that are important to dual eligible beneficiaries considering MA enrollment include:
 - Whether the beneficiary is eligible for medical benefits covered under Medicaid, but not Medicare. Medicaid may provide additional items and services as covered benefits, but Medicaid will only pay for those items and services if they are furnished by Medicaid participating providers. (Note, however, that even providers NOT participating in Medicaid must hold beneficiaries harmless for the cost sharing for Medicare covered benefits.)
 - Whether the beneficiary will need help to find providers who accept both Medicare and Medicaid to obtain any Medicaid benefits.

43. Case Study

Mr. Walsh is a Qualified Medicare Beneficiary (QMB). He enrolls in a Medicare Advantage HMO. Mr. Walsh goes to his primary care doctor to receive a Medicare covered service. The normal copayment is \$25.00. The doctor may only collect from Mr. Walsh any minimal cost sharing allowable under the state Medicaid program which in Mr. Walsh's case is \$2.00 for his physician visit. His doctor may bill the state for the cost sharing, but the hold harmless obligation applies regardless of whether or how much of the cost sharing the state pays.

44. Title Page - Medicare Advantage and Prescription Drugs

45. MA & Prescription Drugs

- An organization offering coordinated care MA plans must offer at least one MA plan with prescription drug coverage (known as an MA-PD plan) in every service area.
- MA PFFS plans have the option of offering prescription drug coverage but are not required to do so.
 - An individual enrolled in an MA PFFS plan that does not include a Part D benefit may enroll in a PDP, even if under the same MA contract, the organization offers another PFFS plan that includes a Part D benefit.
- MA MSA plans are prohibited from offering prescription drug coverage. If an MSA member wants prescription drug coverage, the member must enroll in a stand-alone PDP.

46. MA & Prescription Drugs, continued

- If a beneficiary enrolls in an MA plan that includes Part D prescription drug coverage (an MA-PD plan), the beneficiary can only receive Part D drug coverage through that plan.
- If a beneficiary enrolls in an MA plan that is an HMO or PPO plan that does not include Part D coverage, the beneficiary cannot join a stand-alone Prescription Drug Plan (PDP).
 - Enrollees in certain Employer/Union retiree group plans may have different options.

47. MA & Prescription Drugs: Example

Examples:

Mr. Page enrolled in an MA HMO plan without a prescription drug benefit. Mr. Page's prescription drug costs are increasing quickly, and he wishes to obtain Part D coverage. Mr. Page must wait for an available enrollment period and may either change to a plan offered by his HMO or another Medicare Advantage coordinated care plan that includes drug coverage or disenroll from the HMO and obtain coverage through a PDP and receive his health care benefits from Original Medicare, a PFFS plan or an MSA.

48. Title Page – Other types of Medicare Health Plans

49. Other Types of Medicare Health Plans

There are other types of Medicare health plans, which are NOT Part C or Medicare Advantage plans. The other types of Medicare health plans include:

- Medicare Cost Plans
- Programs of All-Inclusive Care for the Elderly (PACE) plans
- Medicare-Medicaid Plans (MMPs)
- Other Demonstration Plans
 - Other Medicare health plan demonstrations include state-specific demonstrations such as the Minnesota Senior Health Care Options (MSHO) program.

50. Medicare Cost Plans

Medicare Cost Plans are Medicare health plans that are not Medicare Advantage plans, not Part C plans, and are not Original Medicare.

Medicare Cost Plans:

- may offer Part D prescription drug coverage as an optional benefit but are not required to do so.
- may offer other optional supplemental benefits.
- are available only in certain areas in the United States. In 2022 they were offered in some counties in Kansas, Minnesota, North Dakota, South Dakota, Illinois, Wisconsin, Iowa, Missouri, Oklahoma, Wyoming, and Nebraska.

An individual may enroll in a cost plan and a PDP.

- This applies regardless of whether the cost plan offers Part D coverage.

51. Medicare Cost Plans: Network and Out-of-Network Care

Cost plan enrollees can choose to receive Medicare-covered services:

- Under the plan's benefits by going to plan network providers
 - The plan's cost-sharing applies when the enrollee gets services from network providers.
- Under Original Medicare by going to non-network providers
 - Original Medicare cost-sharing applies when the enrollee gets services from non-network providers. This amount is generally higher than the plan cost-sharing.

52. Medicare Cost Plans: Eligibility and Premiums

The following individuals are eligible to enroll in a Medicare cost plan:

- Those with Medicare Parts A and Part B; or
- Those with only Part B. Enrollees with Part B only will not have Part A coverage under the plan unless they purchase it. The plan may adjust the enrollee premium for individuals with Part B only.

Premiums:

- Enrollees must pay their Part B premiums and any plan premium.

53. PACE Plans

Programs of All-Inclusive Care for the Elderly (PACE):

- are Medicare plans for frail, elderly beneficiaries living in the community (i.e., not in a nursing home).
- are available in most states but tend to have small service areas, and thus may only be available in a few counties.
- Offer an adult day health center, where enrollees can get care, meals and other services.
- include comprehensive medical and social service delivery systems using an interdisciplinary team approach in the adult day health center, supplemented by in-home and referral services.

54. PACE Plans, continued

Eligibility for PACE: Participants must be

- age 55 or older.
- reside in the PACE organization's service area.
- be certified as eligible for nursing home care by their state.
- be able to live safely in a community setting at the time of enrollment.

55. Medicare-Medicaid Plans (MMPs)

Medicare-Medicaid Plans (MMPs):

- are established under demonstration authority; and
- are available only in certain counties in Rhode Island, South Carolina, Texas, California, Illinois, Massachusetts, Michigan, New York, and Ohio.
- Only certain individuals eligible for both Medicare and Medicaid may enroll in MMPs.
 - Eligibility varies by state.
- Unlike most D-SNPs, MMPs financially integrate and provide both Medicare and Medicaid benefits.
 - In some states, MMPs may offer additional benefits.
- All MMPs include Part D benefits.
- MMPs are NOT Medicare Advantage plans.

56. Title Page-Enrollee Protections Appeals and Grievances

57. Enrollee Protections

Enrollees of a Medicare Advantage plan or Medicare Cost Plan have a right to:

- file complaints (sometimes called grievances), including complaints about the quality of their care.
- get a decision about health care payment or services, or prescription drug coverage.
- get a review (appeal) of certain decisions about health care payment, coverage of services, or prescription drug coverage.

58. Enrollee Protections: Complaints, Coverage Decisions, Appeals

- Medicare health plan enrollees have two main processes to address concerns or disagreements with their plan.
 - The grievance process is used for complaints about the operations of a plan or its network providers.
 - The appeals process is used to ask for a review of the plan's coverage or payment decisions.

59. Enrollee Protections: Grievances

- Enrollees or their representatives may file a grievance if they experience problems with their health care services such as timeliness, appropriateness, access to, and/or setting of a provided health service, procedure, or item.
- Grievance issues also may include complaints that a covered health service, procedure, or item furnished during a course of treatment did not meet accepted standards for the delivery of health care.
- An enrollee or their representative may make the complaint orally, in writing, or via a CMS website at <https://www.medicare.gov/MedicareComplaintForm/home.aspx>.
- Plans must provide a link to the Medicare.gov website where the enrollee can enter a complaint.

60. Grievance Example

Mr. Russell went to his network doctor for a 2pm appointment. He waited in the waiting room until 3pm when he was finally called back to see the doctor. While waiting, he noticed that the waiting room was dirty. He called his MA plan to complain. The customer service representative assisted him in filing a grievance. The plan investigated his complaint and responded to Mr. Russell.

61. Enrollee Protections: Coverage Decisions

- Coverage decisions are determinations made by a Medicare health plan concerning whether medical care or prescription drugs are covered, how they are covered, and the beneficiary's share of the cost.
- Examples of times when an enrollee may need a coverage decision include:
 - To get prior authorization for a provider to furnish a service.
 - To obtain payment for certain items or services, such as the type or level of services the enrollee thinks should be furnished.
 - To obtain payment for services when the enrollee is temporarily out of the area.
 - To continue a service that the enrollee believes is medically necessary.
 - To obtain payment for a prescription drug.
 - To ask for an exception from a plan's formulary requirements (including step therapy requirements) or tiering structure for prescription drugs.
- An enrollee has a right to ask for prior authorization even when it is not required to find out if a service will be covered by the plan.

62. Enrollee Protections: Appeals

- If an enrollee is not satisfied with the coverage decision, they or in some cases their physician can appeal the decision.
 - Physicians can appeal prior authorization denials on behalf of their patients.
- An appeal is a formal way to ask the plan to review or change a coverage decision.
- An appeal can also be filed if:
 - an enrollee believes a Medicare health plan did not pay for or authorize a service that should be covered.
 - an enrollee believes an authorized service such as a hospitalization or home health care is ending too soon.
 - an enrollee believes a plan has not authorized or paid for a Part D prescription drug that should be covered.

63. Enrollee Protections: Appeals, continued

- Medicare health plans must provide enrollees with a written description of the appeal process.
- Medicare Health Plans offering a Part D benefit must:
 - provide access via a secure website or secure e-mail address on the website for enrollees to quickly request a coverage determination or appeal a decision.
 - require network pharmacies to provide enrollees with a printed notice with the plan's toll-free number and website for requesting a coverage determination.

64. Sources of Additional Information

- General information for organizations currently offering Medicare Advantage plans, or those planning to do so in the future
<http://www.cms.gov/HealthPlansGenInfo/>
- Medicare & You Handbook
<https://www.medicare.gov/medicare-and-you/medicare-and-you.html>
- Detailed information on Medicare Advantage plan requirements, enrollment and eligibility
<https://www.cms.gov/Regulations-and-Guidance/Guidance/Manuals/Internet-Only-Manuals-IOMs-Items/CMS019326.html?DLPage=2&DLEntries=10&DLSort=0&DLSortDir=ascending>